

My response to the NHS consultation

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Q2: Is there anything else we should be considering in producing the mandate to NHS England?

The mandate contains a number of omissions that should be rectified in the final document:

1. The mandate should specifically contain a commitment to ensuring that co-payment prices do not rise above inflation.
2. This mandate has been produced before the comprehensive spending review. This creates a democratic deficit in the very process since financial decisions are being made based on information unavailable to those responding to this consultation. A fresh consultation should be made after the spending review.
3. The mandate does not commit to providing sufficient funding to steer the NHS through the current crisis (see box 4 below). The mandate should guarantee that progressive taxation – the fairest way to provide a comprehensive health system – will be implemented in order to fully support the NHS.

Q3: What views do you have on our overarching objective of improving outcomes and reducing health inequalities, including by using new measures of comparative quality for local CCG populations to complement the national outcomes measures in the NHS Outcomes Framework?

The emphasis on audit culture, monitoring and oversight through outcome measures and comparative metrics sits at odds with the commitment the government has made elsewhere to decreasing regulation. The centralised, big-state accounting procedures that are prioritised here through such procedures will cost a great deal of money and time to implement that could instead be spent on actually doing the job.

Q4: What views do you have on our priorities for the health and care system?

The focus on savings and efficiency are unrealistic and divorced from reality. The document states that:

“This ambition will never compromise the safety or quality of services. It will focus on encouraging the most productive ways of working throughout the NHS, managing demand and maximising income.”

Yet it is clear that the NHS is being systematically starved and that this ambition is already compromising the safety of patients and the quality of services. Richard Murray, director of policy at The King’s Fund, said that “the NHS is in the grip of an unprecedented financial meltdown”. 156 out of 239 trusts are in deficit (or, phrased otherwise: are underfunded). The consultation notes, correctly, that “Research shows the NHS is one of the most efficient health services” but also claims that “we can make better use of the budget”. This is nonsensical. If, by the document’s own admission, one of the most efficient health services is running a £2bn deficit, it cannot make savings and it is under-funded. This unrealistic and damaging commitment to “savings” and “maximising income” should be removed from the mandate. The government should instead commit to sufficiently funding the NHS through progressive taxation.

The mandate should also have a stronger emphasis on retaining good staff. People are crucial to the NHS and the government is losing the confidence of the medical sector and public opinion; especially in light of the junior doctors' strike, being called on an extremely strong mandate. That this is not a priority is troubling.

The removal of the duty to provide comprehensive care is likewise problematic. Coupled with the financial aspects above, it is likely that hospitals will prioritise patients from whom the maximum income ("maximising income") can be extracted. This is at odds with the aim of a health service and the mandate should be changed to ensure this does not happen.